Role of Perceived Social Support and Depressive Symptoms among Mothers of Female versus Male Child within Postpartum Period

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Abstract

Postpartum depression poses a significant challenge to the health of both, the mother and the baby. This study was performed to determine how perceived social support relates to depressive symptoms among mothers during the postpartum phase. Additionally, it investigated potential variations in perceived social support and postpartum depressive symptoms in reference to gender preference. The present study used quantitative cross-sectional research design. Data was collected by using purposive sampling, sample of 50 mothers was collected from hospitals, PIMS Hospital and Begum Jaan Hospital, as well as shopping centres, Centaurus Mall, which comprised of 25 mothers of a male child and 25 mothers of a female child. It included mothers who were all married and had not given birth more than a year ago. Edinburgh Postpartum Depression Scale- Urdu (EPDS-U), Multidimensional Scale of Perceived Social Support- Urdu (MSPSS-U), and Demographic Form in Urdu was utilized for data collection. Results showed moderate negative correlation between perceived social support and depressive symptoms in postpartum period among mothers (r= -.445**). Moreover, results found no significant difference in perceived social support and depressive symptoms among mothers of a female child and a male child. This study concludes that as perceived social support levels increase, there is a decrease at postpartum depressive symptoms. So, the mothers who have high perceived social support are less likely to experience depressive symptoms.

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Chapter 1

Introduction

The turning point in a woman's life, and of all the associated people in her life, is that of when a new life comes into the family. She experiences bundle of joy but at the same time transition into motherhood can have negative influence on the mothers for fulfilling demands of parenting such as the stress for providing child with utmost care, also called as parenting stress. Mothers usually suffer from mental health conditions during pregnancy and after childbirth (Husain et al., 2012). During the time of transitioning into motherhood, women usually express that they feel alone (Lee et al., 2019).

Postpartum Depression (PPD) is defined as a mood disorder. The symptoms experienced by the postpartum women may range from anxiety, loneliness, restlessness, fatigue, insomnia, change in eating habits, feeling of worthlessness, guilt, lack of interest or over interest in baby, to palpitations, hallucinations, delusions, mania, suicidal and homicidal thoughts and loss of touch with reality (Aswathi et al., 2015; Chaudron, 2003). PPD has an adverse negative effect on mother's relationship with the child and further on child's development, and also on the family (Murray et al., 1996).

Social support theory has emerged from the work of Don Drennon-Gala and Francis Cullen. It synthesizes these three types of support - instrumental, informational, and emotional - play a critical role in reducing the likelihood of delinquency and crime, highlighting how supportive societies and relationships can collectively diminish overall delinquency and crime rates, as well as individual engagement in criminal behaviour Similarly, social support plays a crucial role in the development of PPD as mothers might feel rejected, by the relationships surrounding her, or alone in the postpartum period.

Perceived Social Support (PSS) can be defined as how a person perceive support from their social circle and relationships, such as family, friends and others, at the time of need.

Researches indicates how higher PSS significantly contributes to lower depressive symptoms in young adults (Ioannou et al., 2019). Therefore, there is a connection between social support and depression.

Social Support is one of the contributing factors in development of PPD (O'Hara, 2009). The physical, emotional, and hormonal changes along with mental stress, especially after childbirth, of taking care of another human is not emphasized enough in Pakistani Culture. The prevalence of PPD in Pakistan is 63.3% (Piyanee & Arthur, 2009). Some studies also show how cultures also have an effect on the development of PPD (Abdollahi et al., 2011).

In western societies, biologic basis to PPD are more attributed and other risk factors which share similar theme have been noted, such as social support. But in some countries, such as Turkey, India, and China, the impact of sex of the infant also contributed to PPD in different literatures (Goldbort, 2006).

In Pakistan, a patriarchal society, women face immense pressure to bear sons due to the male-dominant culture and the importance placed on carrying the family's lineage and name. This societal mindset results in distress for women, who are often blamed for having daughters, despite this being beyond their control. The pressure begins at a young age, leading to specific expectations regarding foetal sex. Failing to meet these expectations causes stress and depression in women, particularly during the postnatal period, increasing the risk of postpartum depression. Studies quote that daughters are associated with an increased risk of PPD (Rong et al., 2023). These expectations of foetal sex and the birth of “non-preferred baby sex”, which is usually, female in male dominant societies, have been linked with depressive symptoms (Boyce & Hickey, 2005).

The sex of the child is a determinant of PPD and how it actually modified the effect of other risk factors, such as marital violence and hunger in India, has been established through studies (Patel et al., 2002).

Theoretical Framework

Social and Interpersonal Approach

Postpartum Depression, as suggested by researchers, does not have a single theoretical approach but can be a mixture of two or more theories. Lack of social support, from social groups, is described by the social and interpersonal approach of developing PPD. One's own perception of social support being accessible to them, in the stressful conditions, is referred as Perceived Social Support (Taylor, 2011).

Buffering Hypothesis Theory

The Buffering Hypothesis, given by Cohen and Wills, suggests that social support act as a buffer, in stressful scenarios, by preventing the negative consequences of that event on the health, physical as well as mental health. The condition for social support, in this model, is that it must be the one's own perception of social support (Cohen & Wills., 1985).

When a new mother experiences low social support from her support networks, that is, her primary groups, it means she perceives limited assistance in terms of emotional, instrumental, and informational support as well as validation or normalization of her feelings. According to Buffering Hypothesis Theory, if a new mother, receives high social support from her social network, it can shield the mother from experiencing negative impact of the stressful condition and therefore reduce the of severity of PPD. Similarly, if the social support of the mother is low, then it will not be able to protect her against the traumatic experience, therefore, leading to postpartum depressive symptoms.

Sex of the child that the mother has given birth to may decide the level of social support, from social groups, she gets in a male dominant society, such as South Asian

Countries including Pakistan. Pressure of having a son, and then giving birth to a daughter, includes additional stress on top of the experience of giving birth and other responsibilities attached to it. In Pakistan such scenarios can be observed which may lead to postpartum depression. Getting low social support from people around you when a girl is born and high social support when a boy is born, when examined from Buffering Hypothesis Theory, suggests how postpartum depressive symptoms may differ in both aspects.

Transitioning into motherhood brings some stress with the happiness of having a child. All kind of needs of the child have to be fulfilled, providing them with care. This type of parenting stress along with weak quality relationship and support from partner, family, and friends can lead to PPD.

Number of researches suggest social support being a prominent, contributing factor of postpartum depression (Norhayati et al., 2015). Sometimes, low social support from specifically the partner does not mean low Perceived Social Support. This can be noticed in such a case if the PSS from other groups, such as friends and family, is high.

Literature Review

The study on Social Support emerged during the 1970s (Cohen & Syme., 1985). Thereafter, a lot of researches have been done on this topic.

There are different dimensions to social support. Firstly, it can be distinguished as perceived, feeling supported, versus received, reporting of the assistance. Secondly, there are different ways of providing assistance, such as informational, emotional, instrumental. Thirdly, it can be defined by the sources of support, which can be primary social group or secondary social group (Thoits, 2011). Primary social group includes all the personal relationships, such as family, friends and partner. Whereas, secondary social group includes less personal, more regulated and formal relationships, such as schools, other institutes, workplaces etc.

House defines social support as perception and actuality that one is cared for, has assistance available from other people, and is part of a supportive social network (House, 1981). According to this theory, social support has three main aspects: emotional support, informational support, and instrumental support. Emotional support is considered as empathy, love, and trust, while informational support refers to as advice, guidance, and information. Instrumental support means any tangible help, such as financial aid or practical help.

The Main Effect theory and Buffering Hypothesis were given by Cohen and Wills. The Main Effect Theory states that being firmly attached to the social network is beneficial to wellbeing of the person but not necessarily helpful in coping with stressful events. It is more of a general effect of social networks on feelings of stability, and self-worth (Cohen & Wills., 1985).

Individuals with strong social support groups will have fewer physical and mental health problems than those with weaker social support. Thus, it is the quality of the social support group which is important (Stachour, 1998). It is well established that lack of social support affects mental well-being (through anxiety and depression), which then further impairs the physiological processes of the body resulting in adverse health outcomes (Cohen & Syme., 1985).

According to the "buffering hypothesis," social support prevents an individual from negative consequences of stressful events, hence acting as a buffer (Batra et al., 2020).

Zimmet provided the concept of Perceived Social Support, with respect to primary social group, i.e., family, friends, and significant other, by developing a self-report measure of subjectively assessed social support (Zimmet & Dahlem., 1988). Perceived social support suggests that individuals' subjective perception of support is an important factor in determining their well-being.

There is no common consensus among theorists regarding the nature of PPD. Three theoretical perspectives on PPD are majorly discussed. One theory or combination of theories may be suspected for a postpartum depressed woman (Abdollahi et al., 2016).

Biological theories, such as Beck, considered the theoretical bases of PPD as a medical model. It is a pathological mood disorder, not having its root to environmental or social aspects. The goal of her study was to provide support to PPD patients by informing nurses about the consequences of the PPD so that the interventions can be made beforehand (Beck, 2002). Hormonal changes such as estrogenic fluctuations during and after pregnancy predispose women to have depressive symptoms (Soares & Zitek, 2008).

The psychodynamic point of view supports the idea that some unfinished business in women's childhood or family may cause more psychological troubles after birth. Some experts have also noted that childbirth results in loss of mother's identity and leads to withdrawal of love, affection, and loss of independence (Abdollahi et al, 2016).

Cognitive psychology emphasizes on the unrealistic approach towards childbirth and motherhood which in return produces a perfectionist, and controlling behaviour along with certain compulsive action proneness.

Social and interpersonal theory focuses on how uncertainties regarding certain relationships can lead to anxiety and depression (Hammen & Brennan, 2002). Interpersonal factors such as inadequate social support or marital discord can predispose women to PPD (Abdollahi et al., 2016).

Evolutionary approaches suggest that parents will not automatically invest in all offspring and they should reduce or eliminate investment in their children if the costs outweigh the benefits. PPD may be an adaptation that informs mothers that they are suffering or have suffered a fitness cost, which motivates them to reduce or eliminate investment in offspring (Hagen, 1999).

PPD has diverse factors that influence its occurrence, such as culture and others mentioned above. Some researches claims that cultures play an important role in the development of PPD, especially with low rate of PPD being in developed countries and high rate of PPD in developing countries (Abdollahi et al., 2011). Depression can also stem from factors such as unemployment or belonging to low socioeconomic status, in a way that the primary and basic needs are not being met, or that the mother or child has some health condition, mother has obese body image, or the level of education of both parents (Habel et al., 2015).

The preception of having low social support leads to the occurrence of PPD or having depressive symptoms in postpartum period and how other factors, such as the ones discussed above influences it, have been observed over the years in different societies and parts of the world (Cho et al., 2022). In the recent years, many researches concluded how depressive symptoms in postpartum period is associated with perceived social support in Pakistani women. Low levels of social support have been related to more depressive symptoms and how the lack of awarness regarding the the care needed in postpartum, also contributes to it in Pakistan (Jamshaid et al., 2023). The association of other factors related to depression were explored in the studies, which concluded how development of postpartum depression is most importantly affected by lack of support from the family and husband after childbirth and other factors, such as age of the mother, type of delivery, baby feeding habits, and history of being unwell during pregnancy, did not have any significant association with PPD in Pakistan (Riaz et al., 2023).

Rationale

Perceived social support plays a huge role in the susceptibility of mothers who have just gone through the process of giving birth to postpartum depression. There have been several researches that show the prevalence of PPD in Pakistan. Research conducted on the

role of perceived social support in PPD showed a negative correlation between perceived social support and PPD (Jamshaid et al., 2023). In another research, results showed that with one-point increase in global perceived social support, the postnatal depression decreased by 0.2 points in the total sample (Rana et al., 2020). The prevalence of PPD in Pakistan is 63.3% (Klainin & Arthur, 2009).

Although, we have substantial information on perceived social support and postpartum depression with respect to age of the mother, type of delivery, baby feeding habits, and history of being unwell during pregnancy, marital satisfaction, urban and rural areas, and level of education. But there isn't enough information regarding the difference in perceived social support and susceptibility to postpartum depressive symptoms in mother of female versus male child, who is in her postpartum period. The postpartum period is the period that occurs immediately after childbirth. The postpartum period is a critical, yet often neglected period in a new parent's life. According to the World Health Organization (WHO), most maternal and newborn deaths occur in this period, therefore, proper parent and newborn management and care are vital. The postnatal period is generally distributed into three distinct, but continuous phases: acute phase (24hrs following delivery), sub-acute phase (2-6 weeks following delivery), and late phase (6 weeks - 6 months following delivery). The physiological changes happening in the Late Phase are generally very gradual and subtle. Around 10% of women experience perinatal mood and anxiety disorders (PMADs), including depression and anxiety (Romano et al., 2010). This is the reason we are focusing on mothers in their postpartum period in our study.

Son preference is a form of gender bias that persists in many parts of the developing world including Pakistan. This phenomenon has a number of economic, social and cultural causes. It has been found that son preference has a direct effect on status of the mother at home. Women with one or more sons enjoy a higher status while women not bearing sons

face social stigma and pressure from the family. Couples with only daughters and no sons may go for the next child after a shorter interval. Shorter birth spacing leads to higher demand on the mother's body and affects the nutrition and health of both the mother and her existing children. As a result, there is higher risk of disease, malnutrition and mortality, particularly among girl children (Javed & Mughal, 2020).

The key aim of our research is to find the difference in perceived social support susceptibility to postpartum depressive symptoms in mother of female versus male child.

Objectives

The objective of the present study is as follows:

- To examine the relationship between Perceived Social Support and Depressive Symptoms in mothers who are in postpartum period.
- To study the difference between Perceived Social Support and Postpartum Depressive Symptoms in mothers of female versus male child.

Hypothesis

The hypotheses of the present study are:

- There is a negative correlation between perceived social support and postpartum depressive symptoms in mothers who are in postpartum period.
- There is a significant difference in perceived social support and postpartum depressive symptoms in mothers of female child versus male child.

Chapter 2

Method

Research Design

The goal of this research is to find relationship between Perceived Social Support and Postpartum Depressive Symptoms among mothers and to find difference in perceived social support and postpartum depressive symptoms in mothers of female child versus male child. Hence, it will fall under "Cross-Sectional Survey Design".

Sample/Participants

The sample was taken by purposive sampling. Data was collected from 25 mothers of the male child and 25 mothers of the female child, present at hospitals (PIMS Hospital, Begum Jaan Hospital), and shopping centers (Centaurus Mall). The criteria of the sample were women who were married and gave birth not more than a year ago.

Inclusion Criteria

The population of interest for this study were mothers in Rawalpindi and Islamabad. The inclusion criteria were as following.

1. Post-birth duration should not be more than 12 months.
2. All mothers should be married.

Exclusion Criteria

The exclusion criteria are as follows:

1. Mothers with a child of 1 year age or more, were excluded.
2. Single mothers (divorced, widow) were also excluded.
3. Mothers who had babies with severe health problems were excluded.

Operational Definition

Perceived Social Support

Perceived social support refers to how individuals perceive others as sources available to provide material, psychological and overall support during times of need. This variable was operationalized by scoring on Multidimensional Scale of Perceived Social Support (MSPSS) developed by Zimet, Dalhem, and Others in 1988 (Zimet et al., 1988). It measures PSS from friends, family and the significant other.

Postpartum Depression

Postpartum depression (PPD) is a complex mix of physical, emotional, behavioural, and biological changes that happen in some women after giving birth. According to the DSM-5, PPD is a form of major depression that begins within 4 weeks after delivery. The diagnosis of postpartum depression is based not only on the extent of the time between delivery and onset but also on the severity of the depression. This variable was operationalized by scoring on Edinburgh Postnatal Depression Scale (EPDS) designed by Cox, Holden, and Sagovsky in 1987.

Instruments

The following instruments were used to collect data from the selected population.

Demographic Sheet

The demographic sheet was used to collect basic information about the participants. The questions included mother's age, sex of the child, child's age, total number of children, the family type they are living in, child's health, delivery type, breastfeeding, mother's health, marital status, quality time spend with husband, and socioeconomic status. These are the factors that can influence mother's experience with her child and the support she gets from her support groups. All questions were in Urdu language.

Multidimensional Scale of Perceived Social Support (MSPSS) - Urdu

This scale is used to measure the perceived social support that the new mother gets from her family and friends, and significant other. The definition and understanding of significant other is purely subjective. It can be husband, siblings, parents or any other person who plays a significant role in the mother's life. It was originally developed by Zimet, Dalhem, Zimet, & Farley in 1988. Urdu translated version of MSPSS was used, accessed from the work of Sharif and Zaidi, on the Psychometric Validation of MSPSS during Pregnancy in Rural Pakistan (Sharif et al., 2021).

It is a brief scale comprising of 12 items, with 4 items for each subscale. The subscales are family, friends, and partner.

There are seven response categories in this scale, with score ranges from 1 to 7. The scores for all the questions were summed up to get a final score, which indicated the degree of support the participant gets. The total score range is 12-84 which is then divided by the number of items to get final score. Scale score ranging from 1-2.9 was considered low support; a score of 3-5 was considered moderate support; a score from 5.1-7 was considered high support.

The original scale, developed by Zimmet, has high internal consistency reliability with Cronbach's alpha of 0.85–0.91 and good test-retest reliability (Kieu et al., 2023).

The MSPSS- Urdu scale version also showed an excellent internal consistency, yielding a Cronbach's α-value of 0.933. It exhibited an excellent construct validity, and confirmatory factor analysis retained three factors (family, friends, and significant others) for both the depressed and non-depressed Urdu speaking women samples in Pakistan. Internal reliability and construct validity were also established in other multiple studies (Sharif et al., 2021).

Edinburgh Postnatal Depression Scale (EPDS) - Urdu

This scale is a widely used screening tool for detection of postpartum depression. It is a self-report questionnaire. It was designed by Cox, Holden, and Sagovsky in 1987. Urdu translated version of EPDS was used, accessed from the work of Rahman A.

There are total 10 items with four response categories, which are different for each item with scores ranging from 0-3. All the items EXCEPT item number 1, 2 and 4 are reverse scored. Reverse scoring is a method where the responses are given scores in descending order. In this case first response was given "3" and the last response was given "0" score. These scores were then summed up to get a final score. The total score range is 0-30.

Score less than 8 meant depression was not likely; scores 9-11 indicated that depression was possible; score 12-13 indicated high possibility of depression; scores above 14 indicated probable depression. If the participant gives positive score (1, 2, and 3) on item number 10, it indicates that there is suicidality (risk of suicide).

A number of studies that used the Urdu version of the EPDS have found better psychometric properties using a cut-off score of 12 (Husain et al., 2006; see also Dosani et al., 2022). Thereby, 12 was also used as cut-off score.

The EPDS has good reliability, split-half reliability is 0.88 (Hada et al., 2019) and has a Cronbach's α ranging between 0.73 and 0.87 (Dosani et al., 2022).

Data Analysis

The collected data was analysed through Statistical Package for Social Sciences (SPSS) to understand the correlation between both variables of the study and later on T-test was applied to understand trends of the variations of the perceived social support and postpartum depressive symptoms among mothers of a male child and of a female child.

Ethical consideration

The research was conducted in compliance to the APA ethical codes of conduct. Written approval from respective institution, Shifa Tameer-e-Millat University and PIMS Hospital, was taken prior to conduction of research. The participants were given appropriate information about the research and informed consent was obtained from participants. The participants were ensured that the information they provide would only be used for research purposes and would remain confidential including their identity. The participants were also ensured that they are protected and their participation would not cause any harm or trouble for them in any shape. The right to decline or withdraw from participation was given to the participants without any restrictions or consequences.

Procedure

The study sample consists of 50 new mothers, 25 mothers with daughter born and 25 with son born, from Islamabad. The data was collected through in-person interaction with the mothers. The participants were provided with information about purpose of the research, limits of confidentiality, right to decline or withdraw from participation, and how much time it would take to fill the questionnaires, all in Urdu language. After taking written consent, the participants were given the questionnaires (Demographic sheet, MSPSS-Urdu translated, EPDS-Urdu) with proper instructions on how to fill them. It took about 5 minutes to fill the questionnaires. The participants were appreciated for their participation and cooperation. The data collected was then analysed using SPSS to find the correlation between both the variables and to find the group difference between mothers of female child and male child.

Chapter 3

Results

Table 1

Frequency and percentages of study variables (N= 50)

Demographics	M(SD)	*F*	%
Age of mother			
	20-25 years	13	26.0
	26-30 years	21	42.0
	31-35 years	13	26.0
	36-40 years	3	6.0
Sex of Baby			
	Male	25	50.0
	Female	25	50.0
Age of Baby			
	0-3 Months	13	26.0
	3-6 Months	11	22.0
	6-9 Months	11	22.0
	9-12 Months	15	30.0
Total Children			
	1	23	46.0
	2	15	30.0
	3	6	12.0
	4	1	2.0
	5	3	6.0
	6	1	2.0

	9	1	2.0
Family Type			
	Joint	29	58.0
	Nuclear	21	42.0
Baby's Health			
	Healthy	49	98.0
	Disabled	1	2.0
Delivery Type			
	Natural	24	48.0
	Cesarean	26	52.0
Breastfeeding			
	Yes	41	82.0
	No	9	18.0
Mother's Health			
	Healthy	44	88.0
	Unsound	6	12.0
Marital Status			
	Married	50	100.0

Quality Time Spend			
	Yes	47	94.0
	No	3	6.0
Socioeconomic Status	Upper	5	10.0
	Middle	36	72.0
	Lower	9	18.0

Table 1 shows frequency and percentage of study variables of mothers, within postpartum period, with respect to age of the mother, sex of the baby, age of the baby, total children, family type, baby's health, type of delivery, breastfeeding, mother's health, marital status, quality time spend with husband and socioeconomic status. Mothers with age range 26-30 years were 21 (42%), which is more than mothers with age ranges 20-25 years and 31-35 years who were 13 (26%) each, and mothers with age range 36-40 years were 3 (6%). Mothers to a male child were 25 (50%), are equal in number as compared to mothers to a female child, who were also 25 (50%). Children with age range 9-12 months were 15 (30%), which is more than children with age range 0-3 months who were 13 (26%) and children with age ranges 3-6 months and 6-9 months were 11 each (22%). Mothers who had only one child were 23 (46%), which is more than mothers with 2 children, who were 15 (30%). Mothers who had 3 children were 6 (12%), who had 5 children were 3 (6%), and mothers who had 4,6, and 9 children were 1 each (2%). Mothers living in joint family were 29 (58%) which is more than mothers living in nuclear family setting, who were 21 (42%). 49 children (98%) are healthy except 1 child (2%) who is disabled. Mothers who gave birth through cesarean were 26 (52%) which is slightly higher than natural delivery, who were 24 (48%). Mothers who are breastfeeding were 41 (82%) and who are not, were 9 (18%). 44 (88%) mothers are

healthy and 6 (12%) mothers have any health condition. All 50 (100%) mothers are married. 47 (94%) mothers spend quality time with their husbands and 3 (6%) do not spend quality time. Mothers of lower socioeconomic class were 9 (18%), which are less than mothers in middle socioeconomic class who were 36 (72%) and upper socioeconomic class who were 5 (10%)

Table 2

Psychometric Properties of all scales of the study (N=50)

Scales	M	SD	Ranges		Cronbach's Alpha	Kurtosis	Skewness
			Actual	Potential			
MSPSS-U	62.18	15.47	18 - 83	12 - 84	.91	.14	-.92
EPDS-U	11.78	11.78	2 - 27	0 - 30	.83	-.53	.51

Note. *MSPSS= Multidimensional Scale of Perceived Social Support, EPDS= Edinburgh Postnatal Depression Scale, U= Urdu, M= Mean, SD= Standard Deviation*

Table 2 depicts the Cronbach alpha coefficient for both the scales as highly significant. Multidimensional Scale of Perceived Social Support has Cronbach alpha reliability of .917, which is excellent reliability level, and for Edinburgh Postnatal Depression Scale is .832, which is a good reliability level. The kurtosis of 0.14 (MSPSS-U) and -0.53 (EPDS) indicates normal distribution of the data. The skewness of .51 (EPDS-U) shows that the data is symmetrical and the skewness of -.92 (MSPSS-U), negative skewed, shows that the data is slightly skewed. The actual range of MSPSS-U is 18 – 83, whereas its potential range is 12 – 84. The actual range of EPDS-U is 2 – 27, whereas its potential range is 0 – 30.

Table 3

Descriptive Statistics and Correlations for Study Variables

Variables	1	2
Perceived Social Support	-	-.445**
Depressive Symptoms		-

P<0.001**

Table 3 results indicate that there is a significant relationship among the variables. Perceived Social Support has a moderate negative correlation with depressive symptoms (r= -.445). The double asterisks (**) indicate a high level of statistical significance.

Table 4

Mean, Standard Deviation and t- values of mothers with male and female child on Perceived Social Support and Depressive Symptoms (N=50)

Variables	Males (n=25) M	SD	Females (n=25) M	SD	P	t	95% CI LL	UL	Cohen's d
Perceived Social Support	65.44	14.58	58.92	15.93	.13	1.50	-2.16	15.20	0.42
Depressive Symptoms	10.88	6.05	12.68	6.81	.32	-.98	-5.46	1.86	0.27

Note. *CI= Confidence Interval, LL= Lower Limit, UL= Upper Limit*

Table 4 points out t-test result for differences of sex of the child among mothers in correspondence with Perceived Social Support and Depressive Symptoms, in postpartum period. Results accentuate no significant differences in both variables among different sex of the child. The perceived social support and depressive symptoms of mothers of a male child, M=65.44, SD=14.58 and M=10.88, SD=6.057, respectively, and mothers of a female child, M=58.92, SD=15.93 and M=12.68, SD=6.81, had no vast difference. P value of .13 and .32 indicates that the results are not significant.

Chapter 4

Discussion

The current study aimed to examine the relationship between perceived social support and postpartum depressive symptoms in mothers who are in their postpartum period and to study the difference between perceived social support and postpartum depressive symptoms in mothers of female child versus male child.

First hypothesis was related to relationship between perceived social support and postpartum depressive symptoms in mothers who are in their postpartum period. The primary aim is to understand how the level of perceived social support correlates with postpartum depressive symptoms among mothers. The study explores whether higher perceived social support is associated with lower levels of postpartum depressive symptoms and vice versa. Findings of this study, shown in Table 3, reveal that there is a moderate negative correlation between perceived social support and postpartum depressive symptoms. The term "moderate" here indicates that while there is a discernible relationship between perceived social support and postpartum depressive symptoms, it's not an absolute or extremely strong connection. However, it's significant enough to suggest a meaningful association between these variables.

A moderate negative correlation between perceived social support and postpartum depressive symptoms aligns with previous research, reinforcing the understanding that higher perceived social support tends to correlate with lower postpartum depressive symptoms. Mothers who have higher social support during their postpartum period tend to have lesser postpartum depressive symptoms and vice versa. Research confirmed that perceived social support played a moderating role ($\beta = 0.97$, $p = 0.01$) in the relationship between PPD and mental health among Pakistani women. This study concluded that perceived social support has an important role in PPD and the health of Pakistani women (Jamshaid et al., 2023).

The second hypothesis was regarding difference between perceived social support and postpartum depressive symptoms in mothers of female child versus male child. It seeks to determine if mothers of female children experience different levels of perceived social support or postpartum depressive symptoms compared to mothers of male children. This comparison might help uncover any potential correlations or differences influenced by the sex of the child, providing insights into specific factors that might impact postpartum mental health. Findings of this study, shown in Table 4, shows that whether the mother gives birth to a female child or a male child, there is no significant difference between perceived social support they get during postpartum period, and eventually are likely to equally experience depressive symptoms regardless of sex of their baby.

The preference for a male child over a female child has been observed in various Asian countries, deeply rooted in cultural, social, and economic factors. This preference often manifests in societal attitudes and behaviors that favor boys over girls, leading to differential treatment and support for parents based on the gender of their baby, particularly affecting mothers. Compared to mothers of female children, mothers of male children may receive more praise and social support from their families. Due to a lack of social support, mothers who have given birth to female children may be more susceptible to postpartum depression.

Research conducted by Javed showed that the disproportionate preference for sons prevalent among Pakistani households can be clearly seen in the country's skewed sex ratios at birth suggesting that parents with one or more sons are more likely to stop childbearing compared with those who have none (Javed & Mughal, 2020). Another research conducted showed that a significantly increased risk for postnatal depression was associated with having a baby of the undesired sex (Boyce & Hickey, 2005). Research showed that more mothers of boys than of girls scored less than or equal to 12 on the EPDS and significantly more mothers of boys than of girls recalled postpartum sadness (Lagerberg & Magnusson, 2012).

The findings of present study showed that there's no substantial contrast in the perceived social support received by mothers based on the gender of their baby. The potential reasons behind these observations—such as a small sample, unaccounted-for demographic variables, and respondent bias—can significantly influence and sometimes distort research findings. The study's sample size or demographics may not be reflective of the broader population. Variables like education level, cultural background, or access to healthcare services might play a crucial role in shaping the perception of social support and the likelihood of experiencing postpartum depression. Overlooking these variables can limit the accuracy of the conclusions drawn. Research concludes that son preference decreases in couple's level of education. It is more intense among middle-class and rural households (Javed & Mughal, 2022). This finding advocates for the reason why there is no influence of gender preference in our findings. Mothers might consciously or subconsciously provide answers that align with societal expectations or perceived norms. This can impact the accuracy of the data collected and, consequently, the study's findings.

Limitation and Suggestions

- The present study included married women only. Future study can be designed for single mothers (widows, divorced women).

- The data collected for present study was only from Islamabad based on purposive sampling, which effects the generalizability of this study. Future studies should collect data from all over Pakistan, so that the results can be generalized to a larger context.

- Total sample for the present study was only 50. Future studies should increase sample size for more reliability of results.

- The present study did not examine the effect of other demographic variables such as education level, family type and socioeconomic status of participants. It is suggested

that future studies be conducted in order to investigate the effect of these demographic variables.

- This study excluded those mothers whose babies were suffering from any kind of physiological or psychological illness. Future researches can be done targeting those mothers specifically.

Implications

- Findings of this research can be used to develop interventions that specifically target enhancing social support networks for mothers at risk of or experiencing postpartum depression.

- Findings of this research can advocate for policies that encourage workplaces to provide flexible schedules or support programs for new parents.

- Findings of this research can be used to develop programs which offer guidance to expecting parents.

- It provides evidence for the need of conducting campaigns to raise awareness about the importance of social support in combating postpartum depression.

- This research provides a pathway for variety of future researches to address the topic with a different perspective.

Conclusion

The objective of present study was to investigate the relationship between symptoms of postpartum depression and perceived social support in postpartum mothers, with a particular emphasis on potential differences based on the gender of the child. Urdu translated

versions of Multidimensional Scale of Perceived Social Support (MSPSS) and Edinburgh Postnatal Depression Scale (EPDS) were used to collect data from the participants.

The sample consisted of total 50 participants, 25 mothers of a female child and 25 mothers of a male child. The data, using purposive sampling, was collected from hospitals (PIMS Hospital, Begum Jaan Hospital) and a shopping center (Centaurus Mall).

Study results showed that there is a moderate negative correlation between perceived social support and postpartum depressive symptoms in postpartum mothers. The study depicted that mothers who have high perceived social support are less likely to experience postpartum depressive symptoms and vice versa. This study also concludes that there is no significant difference between perceived social support and postpartum depressive symptoms in mother based on sex of the baby.

References

Abdollahi, F., Lye, M. S., Zain, A., Ghazali, S. S., & Zarghami, M. (2011). Postnatal Depression and Its Associated Factors in Women From Different Cultures. *Iranian Journal of Psychiatry and Behavioral Sciences*, *5*(2), 5-11.

Abdollahi, F., Lye, M. S., & Zarghami, M. (2016). Perspective of Postpartum Depression Theories: A Narrative Literature Review. *North American Journal of Medical Sciences*, *8*(6), 232–236. https://doi.org/10.4103/1947-2714.185027

Aswathi, A., Rajendiren, S., Nimesh, A., Philip, R. R., Kattimani, S., Jayalakshmi, D., ... & Dhiman, P. (2015). High serum testosterone levels during postpartum period are associated with postpartum depression. *Asian Journal of Psychiatry*, *17,* 85-88. https://doi.org/10.1016/j.ajp.2015.08.008

Batra, K., Pharr, J., Olawepo, J. O., & Cruz, P. (2020). Understanding the multidimensional trajectory of psychosocial maternal risk factors causing preterm birth: A systematic review. *Asian Journal of Psychiatry, 54,* 102436. https://doi.org/10.1016/j.ajp.2020.102436

Beck, C. T. (2002). Theoretical Perspectives of Postpartum Depression and Their Treatment Implications. *The American Journal of Maternal/Child Nursing, 27*(5), 282-287.

Boyce, P., & Hickey, A. (2005). Psychosocial risk factors to major depression after childbirth. *Social psychiatry and psychiatric epidemiology, 40,* 605-612. https://doi.org/10.1007/s00127-005-0931-0

Chaudron, L. H. (2003). Postpartum Depression: What Pediatricians Need to Know. *Pediatrics in Review*, *24*(5), 154-161. https://doi.org/10.1542/pir.24-5-154

Cho, H., Lee, K., Choi, E., Cho, H. N., Park, B., Suh, M., ... & Choi, K. S. (2022). Association between social support and postpartum depression. *Scientific reports, 12*(1), 3128. https://doi.org/10.1038/s41598-022-07248-7

Cohen, S. E., & Syme, S. I (1985). *Social support and health.* New York: Academic Press.

Cohen, S., & Wills, T. A. (1985). Stress, social support, and the buffering hypothesis. *Psychological bulletin*, *98*(2), 310. https://doi.org/10.1037/0033-2909.98.2.310

Dosani, A., Yim, I. S., Shaikh, K., Lalani, S., Alcantara, J., Letourneau, N., & Premji, S. S. (2022). Psychometric analysis of the Edinburgh Postnatal Depression Scale and Pregnancy Related Anxiety Questionnaire in Pakistani pregnant women. *Asian Journal of Psychiatry, 72,* 103066. https://doi.org/10.1016/j.ajp.2022.103066

Goldbort, J. (2006). Transcultural analysis of postpartum depression. *MCN: The American Journal of Maternal/Child Nursing*, *31*(2), 121-126.

Habel, C., Feeley, N., Hayton, B., Bell, L., & Zelkowitz, P. (2015). Causes of women′ s postpartum depression symptoms: Men's and women'ss perceptions. *Midwifery, 31*(7), 728-734. https://doi.org/10.1016/j.midw.2015.03.007

Hada, A., Kubota, C., Imura, M., Takauma, F., Tada, K., & Kitamura, T. (2019). The Edinburgh Postnatal Depression Scale: Model Comparison of Factor Structure and its Psychosocial Correlates Among Mothers at One Month After Childbirth in Japan. *The Open Family Studies Journal, 11*(1), 1-17.

Hagen, E. H. (1999). The functions of postpartum depression. *Evolution and Human Behavior, 20*(5), 325-359. https://doi.org/10.1016/S1090-5138(99)00016-1

Hammen, C., & Brennan, P. A. (2002). Interpersonal dysfunction in depressed women: impairments independent of depressive symptoms. *Journal of Affective Disorders, 72*(2), 145-156. https://doi.org/10.1016/S0165-0327(01)00455-4

House, J. S. (1981). *Work stress and social support.* Addison-Wesley Pub. Co.

Husain, N., Bevc, I., Husain, M., Chaudhry, I. B., Atif, N., & Rahman, A. (2006). Prevalence and social correlates of postnatal depression in a low income country. *Archives of women's mental health*, *9,* 197-202. https://doi.org/10.1007/s00737-006-0129-9

Husain, N., Cruickshank, K., Husain, M., Khan, S., Tomenson, B., & Rahman, A. (2012). Social stress and depression during pregnancy and in the postnatal period in British Pakistani mothers: a cohort study. *Journal of affective disorders*, *140*(3), 268-276. https://doi.org/10.1016/j.jad.2012.02.009

Ioannou, M., Kassianos, A. P., & Symeou, M. (2018). Coping With Depressive Symptoms in Young Adults: Perceived Social Support Protects Against Depressive Symptoms Only Under Moderate Levels of Stress. *Frontiers in Psychology, 9,* 2780. https://doi.org/10.3389/fpsyg.2018.02780

Jamshaid, S., Malik, N. I., Ullah, I., Saboor, S., Arain, F., & De Berardis, D. (2023). Postpartum Depression and Health: Role of Perceived Social Support among Pakistani Women. *Diseases, 11*(2), 53. https://doi.org/10.3390/diseases11020053

Javed, R., & Mughal, M. (2020). Preference for boys and length of birth intervals in Pakistan. *Research in Economics, 74*(2), 140-152. https://doi.org/10.1016/j.rie.2020.04.001

Javed, R., & Mughal, M. (2022). Changing patterns of son preference and fertility in Pakistan. *Journal of International Development, 34*(6), 1086-1109. https://doi.org/10.1002/jid.3618

Kieu, P. T., Vuong, N. L., & Dung, D. V. (2023). Validation of Multidimensional Scale of Perceived Social Support (MSPSS) in Vietnamese Among People Living with

HIV/AIDS. *AIDS and Behaviour,* 1-9. https://doi.org/10.1007/s10461-022-03974-1

Klainin, P., & Arthur, D. G. (2009). Postpartum depression in Asian cultures: a literature review. *International Journal of Nursing Studies,* 46(10), 1355-1373. https://doi.org/10.1016/j.ijnurstu.2009.02.012

Lagerberg, D., & Magnusson, M. (2012). Infant gender and postpartum sadness in the light of region of birth and some other factors: a contribution to the knowledge of postpartum depression. *Archives of women's mental health, 15,* 121-130. https://doi.org/10.1007/s00737-012-0265-3

Lee, K., Vasileiou, K., & Barnett, J. (2019). 'Lonely within the mother': An exploratory study of first-time mothers' experiences of loneliness. *Journal of Health Psychology, 24*(10), 1334-1344. https://doi.org/10.1177/1359105317723451

Murray, L., Fiori-Cowley, A., Hooper, R., & Cooper, P. (1996). The Impact of Postnatal Depression and Associated Adversity on Early Mother-Infant Interactions and Later Infant Outcome. *Child Development*, *67*(5), 2512-2526. https://doi.org/10.1111/j.1467-8624.1996.tb01871.x

Norhayati, M. N., Hazlina, N. N., Asrenee, A. R., & Emilin, W. W. (2015). Magnitude and risk factors for postpartum symptoms: A literature review. *Journal of Affective Disorders*, *175,* 34-52. https://doi.org/10.1016/j.jad.2014.12.041

O'Hara, M. W. (2009). Postpartum depression: what we know. *Journal of Clinical Psychology*, *65*(10), 1258-1269. https://doi.org/10.1002/jclp.20644

Patel, V., Rodrigues, M., & DeSouza, N. (2002). Gender, poverty, and postnatal depression: a study of mothers in Goa, India. *American journal of Psychiatry*, *159*(1), 43-47. https://doi.org/10.1176/appi.ajp.159.1.43

Rana, M. S., Asif, M., Hanif, A., Gilani, S. A., Habiba, U., Ayaz, S., & Ali, B. (2020). Effect of Perceived Social Support on Postnatal Depression in Females with Cesarean Section and Normal Vaginal Delivery. *RADS Journal of Pharmacy and Pharmaceutical Sciences, 8*(4), 222-230. https://doi.org/10.37962/jpps.v8i4.482

Riaz, N., Mahmood, H., Khan, J., Habib, M. F., Saeed, I., Minhas, A., ... & Raisani, S. A. (2023). Association of Postpartum Depression with Social Support: A Cross-Sectional Study in A Hospital Setting of Rawalpindi-Islamabad, Pakistan: Postpartum Depression with Social Support. *Pakistan Journal of Health Sciences, 4*(05), 02-07. https://doi.org/10.54393/pjhs.v4i06.818

Romano, M., Cacciatore, A., Giordano, R., & La Rosa, B. (2010). Postpartum period: three distinct but continuous phases. *Journal of prenatal medicines,* 4(2), 22.

Rong, X. F., Li, M. Q., Pérez-López, F. R., Wu, J. N., & Zhang, B. (2023). Maternal expectations of fetal gender and risk of postpartum depression. *BMC Pregnancy and*

Childbirth, *23*(1), 112. https://doi.org/10.1186/s12884-023-05419-6

Sharif, M., Zaidi, A., Waqas, A., Malik, A., Hagaman, A., Maselko, J., ... & Rahman, A. (2021). Psychometric validation of the Multidimensional scale of perceived social support during pregnancy in rural Pakistan. *Frontiers in Psychology, 12,* 601563. https://doi.org/10.3389/fpsyg.2021.601563

Soares, C. N., & Zitek, B. (2008). Reproductive hormone sensitivity and risk for depression across the female life cycle: A continuum of vulnerability?. *Journal of Psychiatry and Neuroscience, 33*(4), 331-343.

Stachour, V. M. (1998). The Role of Social Support in Mediating Stress and Illness. *Psychology Commons*, 57.

Taylor, S. E. (2011). Social support: A review. In *The Oxford handbook of health psychology, 1*, 189-214.

Thoits, P. A. (2011). Mechanisms linking social ties and support to physical and mental health. *Journal of health and social behavior, 52*(2), 145-161. https://doi.org/10.1177/0022146510395592

Zimet, G. D., & Farley, G. K. (1988). The Multidimensional Scale of Perceived Social Support. *Journal of Personality Assessment*, *52,* 30-41.

Zimet, G. D., & Farley, G. K. (1988). Perceived Social Support. *Journal of Personality Assessment*, *52*, 30-41.

Appendices

Appendix A

باخبر
رضامندی

یہ تحقیق ہم ان خواتین پر کر رہے ہیں جن کے ہاں گزشتہ ایک سال کی مدت میں بچے کی ولادت ہوئی ہے.

اس تحقیق کا مقصد ان خواتین میں ذہنی دباؤ اور معاشرے سے ملنے والی مدد یا سہارے کے متعلق جانچ کرنا ہے۔

یہ تحقیق شفا تمیر ملت یونیورسٹی، اسلام آباد کے طلباء کی جانب سے کی جا رہی ہے۔

اس تحقیق میں شرکت کے لئےضروری*ہے کہ آپ کےہاں کم از کم ایک سال پہلے بچےکی ولادت ہوئی ہو۔ آپ کی شناخت کے ساتھ آپ کی فراہم کردہ تمام معلومات کو خفیہ رکھا جائے گا اور اسے صرف تحقیقی مقاصد کے لیے استعمال کیا جائے گا۔ اس تحقیق کا حصہ بننا مکمل طور پر رضاکارانہ ہے اور فیصلہ صرف آپ کا ہے۔ آپ کو بغیر کسی پابندی کے کسی بھی موقع پر شرکت سے انکار یا تحقیق سے دستبردار ہونے کا حق حاصل ہے۔ مزید وضاحت اور معلومات حاصل کرنے کے لیے آپ تحقیق میں شامل لوگوں میں سے کسی سے بھی رابطہ کرنے کے لیے آزاد ہیں۔ آپ کے تعاون کا شکریہ*

• تاریخ

• دستخط

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Appendix B

آپ کی عمر کیا ہے؟

(وقت کا بعد کے پیدائش) ہے؟ کیا عمر کی بچے کے آپ

- مہینے 0-3
- مہینے 3-6
- مہینے 6-9
- مہینے 9-12

ہے؟ کیا جنس کی بچے

- بیٹا
- بیٹی

آپ کے کتنے بچے ہیں؟

؟ ہے نظام خاندانی کیا کا آپ

- خاندان مشترکه
- علیدہ خاندان

کیا بچہ صحت مند ہے یا کوئی معذوری ہے؟

- ہے مند صحت
- ہے معذوری

تھی؟ کیا قسم کی پیدائش

- (Natural) طبیعی
- (Caesarean) سیزرین

؟ ہیں رہے پلا دودھ کو بچے اپنے آپ کیا

- ہاں جی
- نہیں

ہیں؟ شکار کا بیماری نفسیاتی یا جسمانی کسی آپ کیا

- ہاں
- نہیں

ہے؟ کیا حیثیت ازدواجی کی آپ

- شدہ شادی
- یافتہ طلاق
- بیوہ

ہیں؟ گزارتے وقت معیاری ساتھ کے بچوں اپنے اور ساتھ کے آپ شوہر کے آپ کیا

- ہاں
- نہیں

ہے؟ کیا حیثیت سماجی کی آپ

- طبقہ اعلی
- طبقہ درمیانہ
- طبقہ نچلا

Appendix C

Edinburgh Postnatal Depression Scale (Urdu Translation)

آپ کیسا محسوس کر رہی ہیں؟

جیسا کہ تھوڑا عرصہ پہلے آپکے ہاں ولادت ہوئ ہے، ہم یہ جاننا چاہیں گے کہ اب آپ کیسا محسوس کر رہی ہیں؟ مہربانی فرما کر اپنے جواب جو تقریبا آپکی حالت کے مطابق ہوں، کے نیچے سطر لگائیں کہ پچھلے سات دنوں سے آپکی طبعیت کیسی ہے نہ صرف یہ کہ آج کیسا محسوس کرتی ہیں۔ ذیل میں پہلے سے حل شدہ مثال دی جا رہی ہے۔

میں نے اپنے آپکو خوش محسوس کیا ہے۔

ہاں، اکثراوقات

ہاں، بعض اوقات

نہیں، بہت زیادہ نہیں

نہیں، بالکل نہیں

اس کا یہ مطلب ہوگا کہ " پچھلے ہفتے کے دوران میں نے بعض اوقات اپنے آپکو خوش محسوس کیا ہے۔" ازراہ کرم باقی سوالات کو اسی طرح مکمل کریں۔

پچھلے سات دنوں سے

1۔ چیزوں کے مزاحیہ رخ کو دیکھ کر میں ہنسنے کے قابل تھی:

- اتنا زیادہ جتنا پہلے ہنس سکتی تھی

- اب اتنا زیادہ نہیں

- یقیناً اب کے اتنا زیادہ نہیں

- بالکل نہیں

2۔ میں خوشی سے کسی چیز کے واقع ہونے کا انتظار کرتی ہوں:

- اتنا زیادہ جتنا پہلے کیا کرتی تھی

- پہلے سے قدرے کم
- یقیناً اس سے کم جتنا پہلے کیا کرتی تھی
- بالکل بھی نہیں

3- جب کچھ غلط ہوجائے تو اپنے میں آپکو الزام دیتی ہوں:

- اتنا ہی جتنا میں پہلے دیا کرتی تھی
- اس سے قدرے کم جتنا پہلے دیا کرتی تھی
- یقیناً اس سے کم جتنا پہلے دیا کرتی تھی
- نہیں بھی نہیں

4- بغیر کسی معقول وجہ کے میں پریشان اور فکرمند رہی ہوں:

- نہیں، بالکل نہیں
- شاید ہی کبھی
- ہاں، کبھی کبھار
- ہاں، اکثر اوقات

5- میں نے بغیر کسی معقول وجہ کے خوف اور گھبراہٹ محسوس کی ہے:

- ہاں، بہت زیادہ
- ہاں، کبھی کبھار
- نہیں، زیادہ نہیں
- نہیں، کبھی نہیں

6- چیزیں میرے سر پر سوار رہی ہیں:

- ہاں، اکثر اوقات میں کئ چیزوں کو نپٹانے میں ناکام رہی ہوں-
- ہاں، کبھی کبھی میں پہلے کی طرح چیزوں کو نپٹانے میں ناکام رہی ہوں-

- نہیں، میں اکثر اوقات چیزوں کو اچھی طرح نہیں نپٹا سکتی۔
- نہیں، میں پہلے کی طرح ہی خوش اسلوبی سے نپٹتی رہی ہوں۔

7۔ میں اتنی ناخوش رہی ہوں کہ مجھے سونے میں مشکل پیش آتی رہی ہے:

- ہاں، زیادہ تر وقت
- ہاں، اکثر اوقات
- بہت زیادہ بار نہیں
- نہیں، بالکل نہیں

8۔ میں نے خود کو غمگین اور افسردہ محسوس کیا ہے:

- ہاں، زیادہ تر وقت
- ہاں، اکثر اوقات
- بہت زیادہ بار نہیں
- نہیں، بالکل نہیں

9۔ میں اتنی زیادہ ناخوش رہی ہوں کہ مجھے روتی رہی ہوں:

- ہاں، زیادہ تر وقت
- ہاں، اکثر اوقات
- صرف کبھی کبھار
- نہیں، بالکل نہیں

10۔ خود کو نقصان پہنچانے کا خیال میرے دل میں پیدا ہوا:

- ہاں، اکثر اوقات
- کبھی کبھار
- شاید ہی کبھی
- کبھی نہیں

Appendix D

Multidimensional Scale of Perceived Social Support (MSPSS)

SECTION 7. MSPSS QUESTIONNAIRE

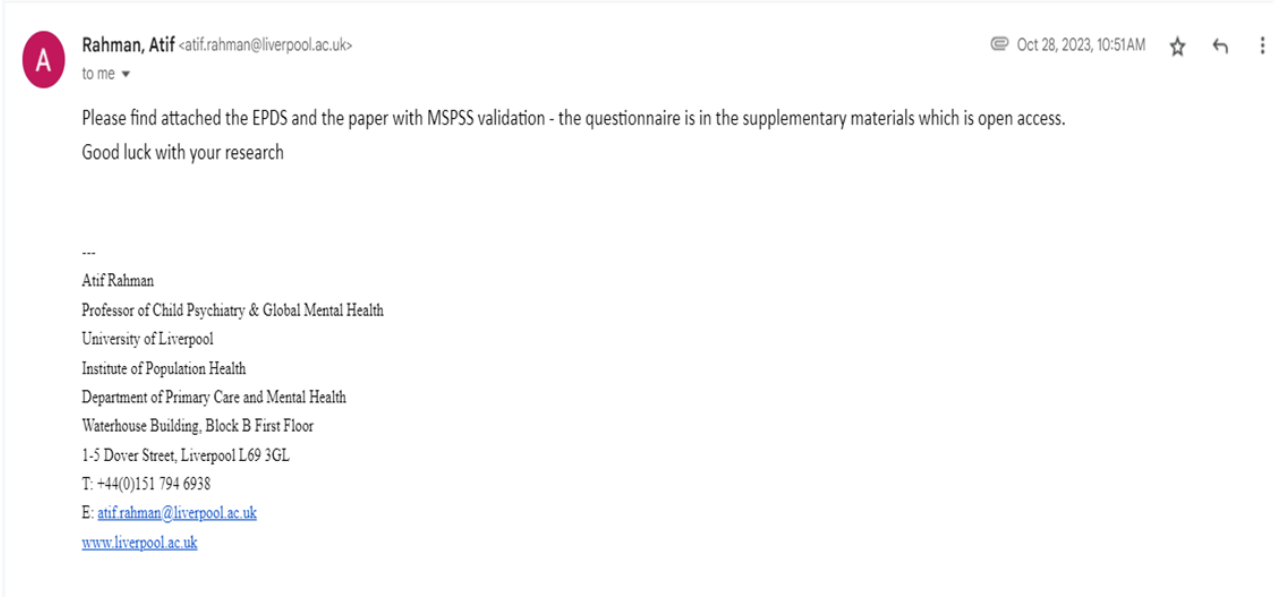
Now I would like to ask you some questions about support you received from people around you. We would ask you to rate the items below against the scale provided.

اب میں آپ سے کچھ سوال ایسے پوچھوں گا جن سے ہمیں اندازہ ہوگا کہ آپ کو اردگرد کے افراد سے کس قدر سہارا حاصل ہے۔ آپ ان سوالات کا جواب اس سکیل کے مطابق بتائیں۔

1. very strongly disagree
2. strongly disagree
3. somewhat disagree
4. neither agree nor disagree
5. somewhat agree
6. strongly agree
7. very strongly agree

1.	There is a special person who is around when I am in need. ضرورت کے وقت کوئی ایسا بھروسہ والا شخص ہے جو آپ کو سہارا دے سکتا ہے۔	1 2 3 4 5 6 7
2.	There is a special person with whom I can share my joys and sorrows. کوئی ایسا شخص ہے جس کا ساتھ اپنا دکھ سکھ کر سکتے ہیں۔	1 2 3 4 5 6 7
3.	I have a special person who is a real source of comfort to me. کوئی ایسا شخص ہے جس سے بات کر کے سکون ملے۔	1 2 3 4 5 6 7
4.	There is a special person in my life who cares about my feelings. میری زندگی میں ایسا بھروسہ والا شخص ہے۔ جو میری طبیعت سمجھ سکتا ہے۔	1 2 3 4 5 6 7
5.	My family really tries to help me. میرے گھر والے میری مدد کرنے کی پوری کوشش کرتے ہیں۔	1 2 3 4 5 6 7
6.	I get the emotional help and support I need from my family. میرے گھر والے مشکل میں میرے ساتھ دکھ سکھ اور مدد کرتے ہیں۔	1 2 3 4 5 6 7
7.	I can talk about my problems with my family. میں اپنے گھر والوں سے اپنی مشکلوں اور مسائل کے بارے میں بات کر سکتی ہوں۔	1 2 3 4 5 6 7
8.	My family is willing to help me make decisions. اگر مجھے کوئی فیصلہ کرنا ہو تو میرے گھر والے مجھے مشورہ دینے کے لئے تیار ہوتے ہیں۔	1 2 3 4 5 6 7
9.	My friends really try to help me. میری دوست اور سہیلیاں میری مدد کرنے کی پوری کوشش کرتی ہیں۔	1 2 3 4 5 6 7
10.	I can count on my friend when things go wrong. اگر مجھے کوئی مشکل ہو تو میری دوست اور سہیلیاں میری مدد کے لئے موجود ہوگی۔	1 2 3 4 5 6 7
11.	I have friends with whom I can share my joys and sorrows. میں اپنی دوست اور سہیلیوں کے ساتھ اپنا دکھ سکھ کر سکتی ہوں۔	1 2 3 4 5 6 7
12.	I can talk about my problems with my friends. اگر کوئی مسئلہ ہو تو میں اپنی دوست اور سہیلی سے اس بارے میں بات کر سکتی ہوں۔	1 2 3 4 5 6 7

Total Score



Rahman, Atif <atif.rahman@liverpool.ac.uk> Oct 28, 2023, 10:51AM

to me

Please find attached the EPDS and the paper with MSPSS validation - the questionnaire is in the supplementary materials which is open access.

Good luck with your research

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